

Department of Pediatric Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 4.6 Yr/M
Admission No.: anonymous	Department Unit: Pediatric Surgery (Ped Sx Unit 1)	Consultant/Faculty: anonymous
Ward: Pedia SX 3C4 HDU Ward	Room/Bed: 3 C 4 HDU/ pea hdu 3	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 13-Mar-2023 12:26	D.O.D. : 01-Apr-2023	D.O. Print Report: 08-Feb-2026 15:44

FINAL DIAGNOSIS:

Anorectal malformation;

OPERATION PERFORMED:

ABDOMINAL PSARP On 22-Mar-2023

OPERATIVE FINDINGS: UNDER GA AAP PRONE POSITION

ANL SPHICTER COMPLEX MARKED LEISON MUSCLE SPHICTER AND BOUNDRIES MARKED

MIDLINE INCION EXTENDING FROM COCCYX AND DEEPENED

SPHICTER MUSCLE SPLIT IN MIDLINE

SOT TISSUE WAWS DISSECTED AND RECTUM WAS OPENING INTO BLADDER OR PROSTATE

LATERAL DISSCETION ADONE 28 FR TUBE PLACED IN CAITY

PT KEPT IN SUPINE POSITION LOWER ABDOMEN HOCKEY STICK INCION MADE AND ABDOMEN SIGMOID MOBILIZE OF LATERALLY

RECTOSIGMOID FULLY MOBILIZED

FISTULA IDENTIFIED AND LIGATED WITH VICRYL 3-0 SUTURE

RECTUM MOBILIZED TO THE PERINEAL INCION AND FIX WITH 4-0 PDS AND ANOPLASTY DONE

INCION WAS CLCLOSED IN LAYERS AND DRESSING DONE PATIENT SHIFTED TO POST OPWITH STABLE VITALS

CHIEF COMPLAINTS:

: K/C/O ARM patient planned for psarp

HISTORY OF PRESENT ILLNESS: : K/C/O ARM

BRIEF HISTORY : No h/o fever. No h/o cold. No h/o cough. No h/o delayed passage of meconium. Sigmoid colostomy done on day 2 of birth.

EXAMINATION FINDINGS :

GENERAL:

PR: 98/M TEMP: 98.2F

RR: 28/MWEIGHT : 11.2 kg

No Pallor, No Cyanosis, No Clubbing, No Generalised LNE, No Generalised Edema

CNS: WNL

CVS: S1S2 HEARD



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



RS: BL AIRENTRY PRESENT

PA: SOFT NON TENDER

LOCAL: Sigmoid colostomy functional.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
22-Mar-2023 19:59	BIOPSY	textbox1	Refer Reports*	-
(HISTOPATHOLOGY)				

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: USS ABDOMEN (16/01/2023): No significant abnormality detected.

DISTAL LOOPOGRAM (20/01/2023): In a known case of ARM, the present loopogram reveals blind rectum with recto-vesical fistula.

HOSPITAL COURSE: patient was admitted in department of pediatric surgery and all routine investigation send and planned for psarp and OPEARTION WAS PERFORMED AND POD 2 PATIENT STOMA WAS FUNCTIONAL AND PATIENT WAS ALOOW ORAL FEED AND TOLEATING ORAL FEEDS WOUND WAS HEALTHY SO PATIENT WAS PLANNED FOR DISCHARGE

CONDITION AT DISCHARGE: : PATIENT IS HEMODYNAMICALLY STABLE. PASSED URINE STOOLS. TOLERATING ORALS AND STOMA FUNCTIONAL AND WOUND IS HEALTHY

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 07-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE :

Summary Prepared By

anonymous

Summary Approved By

anonymous